

Question 3: For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

One of the biggest challenges I see is the uncertainty about accountability when AI influences clinical decisions but is not regulated as a medical device. For example, risk scores or AI-generated summaries may shape how urgently a patient is evaluated or what information a clinician focuses on, yet it is often unclear who is responsible if the tool is wrong or misleading. This creates confusion around liability, standards of care, and how much clinicians are expected to rely on or override these tools.

Privacy, security, and secondary use of clinical data are also real concerns, especially when AI features are embedded into required workflows and cannot easily be turned off. Clinicians and patients may not always know when AI is being used, what data it relies on, or how outputs are generated.

HHS could help by setting clearer expectations for disclosure, so clinicians and patients know when AI is involved and for what purpose. Basic governance norms, such as clearly identifying who approves a tool, who monitors its performance, and who has the authority to pause or roll it back if problems arise, would reduce uncertainty and support safer adoption.

Question 4: For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care?

From a clinician's perspective, evaluation cannot stop once a tool goes live. Before deployment, AI tools should be tested on populations that reflect real patients, not just ideal datasets. This includes understanding how performance varies by factors such as race, language, disability, or care setting. Workflow testing is also critical. Tools should be observed in real clinical environments where outputs are reviewed but not acted upon, to see how they affect decision-making and workload.

After deployment, ongoing monitoring matters just as much. In practice, useful signals include whether alerts fire too often, how frequently clinicians override recommendations, whether performance changes over time, and whether certain groups experience more errors or missed care. HHS could support this by encouraging practical, real-world evaluation and offering simple, non-punitive ways for clinicians and organizations to report safety concerns or unintended consequences.

Question 5: How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

AI adoption would be safer if there were more consistent expectations for independent evaluation. A basic certification approach for non-medical AI device used in clinical care that

focuses on transparency, safety monitoring, and bias assessment, would help clinicians understand what has been reviewed and what has not.

Plain-language “AI labels” would also be helpful. Similar to nutrition labels, these could briefly explain what a tool is designed to do, where it performs well or poorly, and how it is intended to be used. This would help clinicians and patients compare tools and set realistic expectations.

Question 6: What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

As a clinician, AI works best when it clearly saves time or improves reliability. For example, by reducing documentation burden or flagging clinical risk in ways that are paired with clear next steps. It tends to fall short when tools generate frequent, non-actionable alerts, add extra clicks, or provide recommendations without enough context for clinicians to judge whether they are appropriate.

The most promising use cases I see are the early detection of patient deterioration when paired with clear escalation pathways, medication safety support, language access tools for patients with limited English proficiency (with safeguards to verify accuracy), and care coordination tools that help identify and support high-risk patients across settings.

Question 9: What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

From my position, patients and caregivers want AI to make care safer, faster, and better coordinated especially during handoffs and transitions. The main concerns are about privacy, lack of transparency, biased or incorrect outputs, and difficulty correcting errors when AI plays a role in decisions. Clear communication about when AI is used, appropriate patient notice, and accessible ways to raise concerns or request review would help build trust.

Conclusion

From an individual clinician’s perspective, responsible AI adoption depends less on technical sophistication and more on clarity, transparency, and ongoing oversight. Clear disclosure, practical evaluation standards, and safeguards that protect both patients and clinicians would help ensure AI delivers real benefits without shifting risk onto those providing or receiving care.

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